

56% would help. 31% don't know how.

Most of what lands in your inbox this month is a ribbon and an encouragement to start the conversation. This is three things you can actually do before September ends — no budget, no vendor, no meeting.

56% / 31%

More than half of Americans would help a colleague struggling with substance use. Nearly a third don't know what to do.

Addiction Policy Forum & Gallup, May 2026

That gap is the whole problem, and it isn't indifference. It's the absence of a script.

Meanwhile **1 in 10 employees** is living with an untreated substance use disorder and **4 in 5 of the people affected are never treated** — not because treatment doesn't exist, but because the path to it runs straight through the thing people fear most, which is being seen.

All three actions below aim at the same target. **Lower the cost of being seen.**

THREE THINGS YOU CAN DO THIS MONTH

COST — ONE EMAIL

Fix the bar before you fix the policy

If the alcohol is at a staffed bar and the alternative is a warm can of soda on a side table, you've already told the room which choice is the real one.

Stock two or three genuinely good non-alcoholic options. Same bar, same menu, same eye level, offered first rather than as a consolation.

Nobody discloses anything. The signal isn't the drink — it's that somebody thought about it in advance.

This week: send this to whoever plans your next company event.

COST — ONE INTRANET EDIT

Make help findable without a conversation

30% of Americans don't know where to find accurate information about substance use disorder, and the people most likely to need it are the least likely to walk into HR and ask.

So put it where it can be found silently: the SAMHSA National Helpline (1-800-662-HELP), findtreatment.gov, and 988 for crisis — on the intranet home, the benefits portal, the back of the break room door.

No form, no disclosure, no manager in the loop.

This week: one intranet edit and one printed sign.

COST — FIFTEEN MINUTES

Give your managers one sentence

The people who don't know what to do are mostly your people leaders. They notice something, worry about saying the wrong thing, and say nothing until it becomes a performance conversation.

"I've noticed [specific, observable, work-related thing]. I'm not asking you to explain anything to me. I want to make sure you know what's available, and that using it won't cost you your job."

Describe behavior, never diagnose. Don't speculate about cause. Don't promise confidentiality you can't deliver.

This week: fifteen minutes on an agenda that already exists.

Every one of those three actions aims at the left end of this chain.

\$15,000+ STAGE 1 A missed shift Lost output per year. ≈8 hrs/week reduced output × avg U.S. hourly wage (BLS, 2026).	\$20,000+ STAGE 2 A relationship, quietly strained Per leave event. 12-week FMLA: continued premium (KFF, 2025) + backfill (BLS, 2026).	\$30–45K STAGE 3 A screening they didn't pass To replace one employee. 6–9 months of salary in recruiting, onboarding, ramp (SHRM).	\$47,316 STAGE 4 An accident on the floor Average lost-time workers' compensation claim (NCCI, via National Safety Council, 2025).	\$485,320 STAGE 5 A body that's had enough Average cost of one cancer diagnosis to a self-funded plan (Voya Stop-Loss, 2025).
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Someone just had to catch stage one.

Read it right to left and it's a cost problem. Read it left to right and it's a person having a hard year that nobody asked about.

FOR THE PEOPLE WHO ADVISE EMPLOYERS

Employee benefits

The EAP is not the answer here and your clients likely suspect it. At the next renewal, ask what EAP utilization was last year for any reason. Then ask what share of it was substance-related. The silence after the second question is the conversation worth having.

Risk and safety

Substance use is tied to 65% of on-the-job accidents, and 10–20% of workplace fatalities involve alcohol or drugs. Heavy drinkers are 30% more likely to file a workers' comp claim. Your clients already pay for this — as incidents rather than as a program.

For both: the highest-leverage thing you can do this month is read a client's drug and alcohol policy the way an employee would read it. Most describe consequences in detail and support in one line, or not at all. Adding a single sentence about voluntarily seeking help — before an incident, without penalty — costs nothing and changes what the whole document communicates.

Sources: Addiction Policy Forum & Gallup, *Addiction Stigma in America*, May 2026 · SAMHSA NSDUH · NSC/NORC Substance Use Cost Calculator · NCADD · U.S. Dept. of Labor · RAND · BLS 2026 · KFF 2025 · SHRM · NCCI via National Safety Council 2025 · Voya Stop-Loss Analysis 2025.

Most people reading this are not at the finish line

Recovery Month tends to celebrate the people who made it all the way through, and that's worth doing. But most of the people reading this are somewhere else — cutting back, wondering, worrying about someone at home, or three weeks into something they haven't told anyone about.

Sobriety is personal. Whatever that looks like — total sobriety, recovery, sober curious, cutting back, or supporting someone you love — the work of a recovery-friendly workplace is making sure none of it has to be a secret.

Three things. One month. No budget.

Jess Ferretti · VP of Sales, Sobrynth
If you advise employers, forward this to three clients. The three things work whether or not we ever speak.